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GUIDE 03 · THE FLORA COLLECTION

The Hormone Type Guide

Understand your unique hormonal blueprint.

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Not All Hormonal Imbalances Are the Same

Two women can have identical symptoms — fatigue, weight gain, mood swings, irregular cycles — and have completely opposite hormonal pictures. One might have estrogen dominance; the other, low estrogen. The same supplement that helps one would worsen the other. This guide gives you the tools to identify your unique hormonal type and design a protocol that actually fits your biology.

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There are six primary hormone types identified in functional medicine, each requiring a distinct nutritional and supplementation approach. Generic women's hormone protocols fail because they ignore this fundamental variability.

HOW TO USE THIS GUIDE

1. Complete the Hormone Type Quiz on pages 4–5 — be honest about current symptoms.
2. Read your full Hormone Type profile including your specific drivers and risks.
3. Follow the targeted protocol for your type in Section 03.
4. Reassess after 6–8 weeks — many women shift types as they begin to rebalance.

KEY TAKEAWAY

Your hormone type is not fixed. It reflects your current state — shaped by stress, diet, age, and lifestyle. The goal is to move toward the Balanced Type by addressing your specific root causes.

Understanding the Hormone Type System

The six hormone types are defined by the pattern of imbalance present — which hormones are high, low, or dysregulated relative to each other. Most women have a primary type and a secondary type, often driven by the same root cause.

TYPE 1 ESTROGEN DOMINANT

High estrogen relative to progesterone. The most common type. Driven by poor liver detox, gut dysbiosis, xenoestrogen exposure, and chronic stress.

TYPE 2 LOW ESTROGEN

Insufficient estrogen production or accelerated clearance. Common peri- and post-menopausally, but also in over-exercising, under-eating athletes and those under extreme stress.

TYPE 3 LOW PROGESTERONE

Progesterone deficiency without necessarily elevated estrogen. Often presents as luteal phase defect, anxiety, and difficulty sleeping in the second half of the cycle.

TYPE 4 HIGH CORTISOL / ADRENAL DOMINANT

Chronic stress drives sustained cortisol elevation, which steals pregnenolone from sex hormone pathways and suppresses the entire reproductive hormone cascade.

TYPE 5 ANDROGEN EXCESS

Elevated testosterone and/or DHEA, driving acne, unwanted hair growth, irregular cycles, and often underlying insulin resistance (PCOS pattern).

TYPE 6 THYROID & METABOLIC

Thyroid dysfunction (hypo or hyper) disrupts the entire metabolic-hormonal axis, affecting menstrual regularity, weight, mood, and every other system.

The Hormone Type Quiz

Check all symptoms you experience consistently (not just occasionally). Count your checks in each section. Your primary type has the highest score. If two sections are close, you have a dual-type pattern.

TYPE 1 · ESTROGEN DOMINANT

- ☐ Heavy, prolonged periods (>7 days or >80ml)
- ☐ Breast tenderness or fibrocystic breasts
- ☐ PMS: bloating, mood swings, irritability
- ☐ Weight gain around hips, thighs, and lower abdomen
- ☐ Uterine fibroids or endometriosis diagnosis
- ☐ Migraines before or during your period

MY SCORE: ____

TYPE 2 · LOW ESTROGEN

- ☐ Hot flushes or night sweats (any age)
- ☐ Vaginal dryness or pain during intercourse
- ☐ Irregular or absent periods
- ☐ Mood low, anxious, or depressed
- ☐ Joint pain or poor skin elasticity
- ☐ Memory issues, brain fog, poor concentration

MY SCORE: ____

TYPE 3 · LOW PROGESTERONE

- ☐ Spotting 1–5 days before period starts
- ☐ Anxiety, panic attacks, or nervousness in luteal phase
- ☐ Difficulty falling or staying asleep, especially days 15–28
- ☐ Short cycles (less than 25 days)
- ☐ History of miscarriages or fertility challenges
- ☐ Premenstrual migraine or headaches

MY SCORE: ____

TYPE 4 · HIGH CORTISOL / ADRENAL

- ☐ Persistent fatigue that doesn't improve with sleep
- ☐ Belly fat accumulation despite healthy diet
- ☐ Wired but tired — alert at night, sluggish in the morning
- ☐ Frequent illness or slow recovery
- ☐ Salt and sugar cravings
- ☐ Anxiety, low stress resilience, emotional reactivity

MY SCORE: ____

TYPE 5 · ANDROGEN EXCESS

- ☐ Acne on jawline, chin, and back
- ☐ Unwanted hair growth (face, stomach, chest)
- ☐ Irregular or skipped periods
- ☐ Hair thinning or androgenic alopecia
- ☐ PCOS diagnosis or suspected
- ☐ Low mood, depression, poor motivation

MY SCORE: ____

TYPE 6 · THYROID & METABOLIC

- ☐ Unexplained weight gain or difficulty losing weight
- ☐ Persistent fatigue, even with adequate sleep
- ☐ Cold hands, feet, or feeling cold when others are comfortable
- ☐ Hair loss, dry skin, brittle nails
- ☐ Constipation or very slow digestion
- ☐ Low mood, poor memory, brain fog

MY SCORE: ____

Estrogen Dominant Type

Estrogen dominance is the most common hormonal pattern in modern women. The goal is to support estrogen clearance through the liver and gut while reducing the sources of estrogen excess — both internal production and external exposure.

PRIORITY FOODS

- Cruciferous vegetables daily: broccoli, kale, Brussels sprouts (DIM + I3C)
- Ground flaxseeds 2 tbsp/day — lignans block excess estrogen receptors
- Raw carrots daily — unique fibre binds and removes spent estrogen
- Fermented foods — restore estrobolome bacteria for estrogen clearance
- Green tea 2 cups/day — EGCG supports Phase I liver detox

TYPE 1 SUPPLEMENT STACK

- **DIM (Diindolylmethane)**
200–400 mg with food *Shifts estrogen toward safer 2-OH metabolites*
- **Calcium D-Glucarate**
500 mg twice daily *Blocks beta-glucuronidase; prevents estrogen re-absorption*
- **Magnesium Glycinate**
300–400 mg nightly *Supports Phase II liver detox and progesterone balance*
- **B-Complex (methylated)**
1 daily with breakfast *COMT enzyme support for estrogen methylation pathway*

LIFESTYLE PRIORITIES

- Eliminate BPA plastics, conventional cosmetics, and synthetic fragrances
- Prioritise 7–9 hours sleep: liver detox activity peaks between 1–3am
- Reduce alcohol to zero for 8 weeks minimum — alcohol blocks estrogen clearance
- Increase dietary fibre to 35g/day to ensure daily estrogen excretion via stool

Low Progesterone Type

Progesterone deficiency is almost always rooted in either poor corpus luteum function, chronic stress (cortisol steals its precursor), or nutritional deficiencies in zinc and B6. The fix must address all three simultaneously.

TYPE 3 SUPPLEMENT STACK

- **Vitex Agnus-Castus**
400 mg in the AM *Stimulates LH secretion and supports corpus luteum*
- **Zinc**
25–30 mg with food *Essential co-factor for corpus luteum progesterone synthesis*
- **Vitamin B6**
50–100 mg with food *Required for progesterone receptor sensitivity*
- **Magnesium Glycinate**
400 mg nightly *Calms GABA receptors sensitised by progesterone fluctuations*

TYPE 4 PROTOCOL · HIGH CORTISOL

Adrenal Dominant Type

The adrenal dominant type requires the most lifestyle intervention of all six types. Supplements alone will not fix a chronically overactivated stress response. Nervous system downregulation is the primary treatment — supplements are secondary.

TYPE 4 SUPPLEMENT STACK

- **Ashwagandha (KSM-66)**
300–600 mg *Reduces cortisol by 25–30%; improves stress resilience*
- **Phosphatidylserine**
400 mg with dinner *Blunts cortisol response to stress; supports sleep*
- **Rhodiola Rosea**
200–400 mg AM *Adaptogen; improves HPA axis regulation and fatigue*
- **Vitamin C**
1000–2000 mg daily *Adrenal glands have the highest vitamin C concentration in the body*

❑ ADRENAL TYPE · DO NOT

- Do not start intense exercise programmes until cortisol is normalised
- Do not use stimulant supplements (high-dose B12, guarana, high caffeine)
- Do not restrict calories — caloric deficit is a major cortisol trigger
- Do not ignore sleep quality — untreated sleep apnoea perpetuates the cycle

Androgen Excess Type (PCOS Pattern)

Androgen excess in women is almost universally driven by insulin resistance signalling the ovaries to overproduce testosterone. The single most effective intervention is aggressive blood sugar management — more impactful than any supplement.

TYPE 5 SUPPLEMENT STACK

- **Inositol (Myo + D-Chiro, 40:1)**
4g Myo + 100mg D-Chiro daily *Reduces androgens and improves ovarian function in PCOS*
- **N-Acetyl Cysteine (NAC)**
600 mg twice daily *Comparable to metformin for insulin sensitisation in PCOS*
- **Spearmint Tea**
2 cups daily *Reduces free testosterone via anti-androgenic activity*
- **Zinc**
25–30 mg with food *Inhibits 5-alpha reductase; reduces androgen conversion*

TYPE 6 PROTOCOL · THYROID & METABOLIC

Thyroid & Metabolic Type

Thyroid dysfunction requires medical confirmation via blood testing before supplementing. Subclinical hypothyroidism is frequently missed by standard TSH testing — request Free T3, Free T4, reverse T3, and thyroid antibodies.

TYPE 6 SUPPLEMENT STACK

- **Selenium**
200 mcg daily *T4-to-T3 conversion; reduces thyroid antibody levels*
- **Iodine (food-first)**
Via seaweed, fish — not supplemented without testing *Thyroid hormone synthesis; excess iodine can worsen autoimmune thyroid*
- **Iron (if deficient)**
Per blood test results *Iron deficiency impairs thyroid peroxidase enzyme function*
- **Vitamin D3**
2000–5000 IU with K2 *Thyroid receptor co-factor; very commonly deficient in thyroid types*

What to Test and When

Blood tests confirm what the quiz identifies. These are the key markers to request, organised by type. Most can be requested via your GP, or through private labs if your GP declines. Test on cycle day 3 for estrogen and FSH; cycle day 21 for progesterone.

ESSENTIAL PANEL · ALL TYPES

- FSH (Day 3)
- LH (Day 3)
- Estradiol E2 (Day 3 + Day 21)
- Progesterone (Day 21)
- Total & Free Testosterone
- SHBG (Sex Hormone Binding Globulin)
- DHEA-S

THYROID PANEL · TYPE 6

- TSH
- Free T3
- Free T4
- Reverse T3
- Anti-TPO antibodies
- Anti-thyroglobulin antibodies
- Ferritin (iron stores)

METABOLIC PANEL · TYPE 4 & 5

- Fasting glucose
- Fasting insulin
- HbA1c
- Cortisol (AM + PM saliva or blood)
- DHEA-S
- Full blood count
- Vitamin D (25-OH)

At-a-Glance Protocol Summary

Type 1 Estrogen Dom.	SUPPLEMENTS DIM, Calcium D-Glucarate, Magnesium	PRIORITY FOODS Cruciferous daily, flax, raw carrot
Type 2 Low Estrogen	SUPPLEMENTS Phytoestrogens, Vitamin D3, Omega-3	PRIORITY FOODS Fermented soy, flax, seeds
Type 3 Low Progesterone	SUPPLEMENTS Vitex, Zinc, B6, Magnesium	PRIORITY FOODS Zinc-rich foods, pumpkin seeds
Type 4 High Cortisol	SUPPLEMENTS Ashwagandha, Phosphatidylserine, C	PRIORITY FOODS Reduce caffeine, no refined sugar
Type 5 Androgen Excess	SUPPLEMENTS Inositol, NAC, Spearmint, Zinc	PRIORITY FOODS Low GI diet, no dairy, no refined carbs
Type 6 Thyroid/Metabolic	SUPPLEMENTS Selenium, Vitamin D3, Iron (if low)	PRIORITY FOODS Selenium-rich Brazil nuts, seaweed

KEY TAKEAWAY

Most women's hormone type shifts within 3–6 months of targeted intervention. Reassess with the quiz every 8 weeks, and retest bloods every 3 months to track objective improvements. The goal is always the Balanced Type.

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The Flora Collection

01 · The 28-Day Hormone Reset

02 · The Cycle Syncing Meal Bible

03 · The Hormone Type Guide

04 · The Symptom Decoder

05 · The PMS Elimination Guide

06 · The Cortisol Reset

07 · The Cycle Fitness Guide

08 · The 90-Day Rebalancing Roadmap

For educational purposes only. Not medical advice.